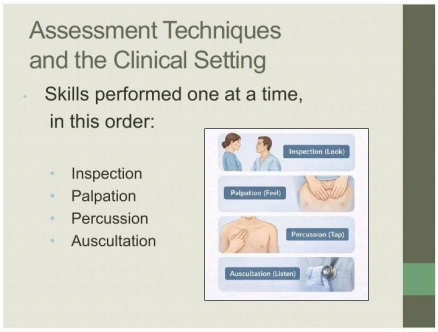
Head-to-Toe Physical Exam: Step by Step (v2)

NRSG 233 Health Assessment, LLU, Fall 2026. This is a personal study and practice guide, not an assignment.

Sources: Her NRSG 233 slides (Module 1 Intro, Mental Status, Module 2 HEENT, Module 3 Skin/Hair/Nails + Braden handout, Module 4 Abdomen & MS) and the Jarvis page references on her HA documentation sheets (HA #1, Module 3, Module 4) take priority. The technique steps and sequence follow Jarvis, *Physical Examination & Health Assessment* (9th ed.), Ch. 28 "The Complete Health Assessment: Adult" (pp. 769–780) and each chapter's Summary Checklist, cited as [Jarvis p. N]. Photos labeled "Jarvis" are textbook photos. Every other photo is from her slides.
Key: IPPA = Inspect, Palpate, Percuss, Auscultate. For the abdomen the order is **I-A-P-P**. Where her slides and Jarvis disagree, go with her slides, and check her check-off sheet when you get it.

1. Prep: before you touch the patient

Check-off (syllabus): *63 pts, Week 9 (Nov 16–27). You do a complete head-to-toe on a partner, and it's recorded.* ***15-minute limit.*** *Prompting isn't allowed (−1 pt each time). You need* ***76%*** *to pass the course. Order: her slides use I-P-P-A, abdomen uses I-A-P-P. Positions: sitting, then supine, then sitting, then standing [Jarvis p. 769].*



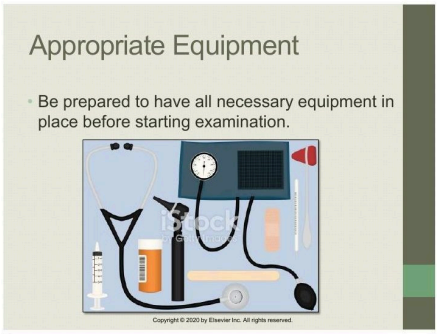
Her Module 1 slides

Technique order: I-P-P-A

Do: Perform the skills one at a time, in order: Inspection, Palpation, Percussion, Auscultation. Palpate light before deep. Percussion tones: tympany, hyperresonance, resonant, dull, flat.
Technique: IPPA
Normal: N/A

> **Say:** N/A

[Her Module 1 slides]



Her Module 1 slides: Appropriate Equipment

Gather equipment, perform hand hygiene

Do: Set out everything first: stethoscope, BP cuff, thermometer, pulse ox, penlight, otoscope/ophthalmoscope, Snellen chart, tongue blade, cotton ball, reflex hammer, gloves, watch with a second hand.
Technique: Wash or foam in, in front of the patient. Her Standard Precautions slide: hand hygiene before and after care, PPE as needed.
Normal: N/A

> **Say:** N/A

[Her Module 1 slides; Jarvis p. 769]


Jarvis p. 770 (textbook photo)

Introduce yourself, identify the patient, provide privacy, explain

Do: Give your name and role. Check two identifiers (name + DOB) against the armband/chart. Close the door or curtain. Explain what you'll do and get consent. Raise the bed to a working height.
Technique: Sit facing the patient while they're still in street clothes. This builds rapport while you take the history.
Normal: Patient is ID'd correctly and agrees to the exam.

> **Say:** "Hi, I'm Luke, a nursing student. Can you tell me your full name and date of birth? I'm going to do a head-to-toe check today. I'll explain each step as I go, and you can ask me to stop anytime."

[Jarvis p. 770]

2. General survey and vital signs

Start the general survey the moment you walk in, and keep watching during the history [Jarvis p. 770].



Jarvis p. 770

General survey

Do: Observe: appears stated age, level of consciousness, skin color, nutrition, posture/position, deformities, mobility (gait, assistive devices, ROM, involuntary movement, rises easily), facial expression, mood/affect, speech, hearing, hygiene.
Technique: Inspection only.
Normal: Alert and oriented, looks stated age, erect posture, steady gait, clear speech, appropriate dress and hygiene, no distress.

> **Say:** (Just talk with them while you observe.) "How are you feeling today?"

[Jarvis p. 770]



Jarvis p. 770

Measurements

Do: Weight, height, waist circumference, BMI. Snellen vision if part of your exam.
Technique: Scale, stadiometer, tape measure.
Normal: BMI 18.5–24.9.

> **Say:** "Please step on the scale and stand tall against the wall for me."

[Jarvis p. 770]



Jarvis p. 771

Vital signs

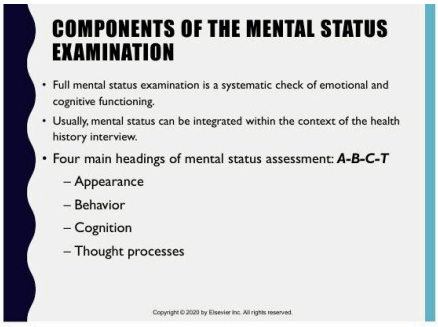
Do: Radial pulse, respirations, BP (patient rested at least 5 min), temp, SpO2, pain (PQRST). Then have them empty their bladder, put on a gown, and sit on the edge of the bed.
Technique: Count the pulse for 30 sec ×2 (60 sec if irregular). Count respirations without telling them. Use the right cuff size, arm at heart level.
Normal: Adult: HR 60–100, RR 12–20, BP <120/<80, SpO2 95–100%, temp ~97–99.5°F. Use her cheat sheet ranges for the test.

> **Say:** "I'm going to check your pulse and blood pressure. Just relax, keep your feet flat, and don't talk for a minute."

[Jarvis p. 771]

3. Mental status

Mostly done while you take the history. Use the A-B-C-T framework [Jarvis p. 79].



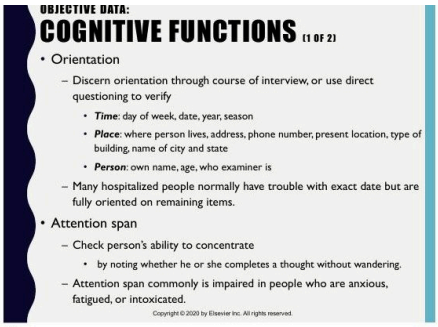
Her Mental Status slides: A-B-C-T

Appearance, Behavior, Cognition, Thought

Do: Appearance: posture, body movements, dress, grooming. Behavior: LOC, facial expression, speech (rate, clarity, word choice), mood and affect. Cognition: orientation (person, place, time, situation), attention span, recent and remote memory, judgment. Thought: process (logical?), content, perceptions, suicide screen.
Technique: Inspect and interview. Use the MMSE if indicated.
Normal: A&O×4, calm, cooperative, mood matches affect, speech clear and goal-directed, memory intact, good judgment, denies thoughts of self-harm.

> **Say:** "Can you tell me where we are and what today's date is? What did you have for breakfast? Have you had any thoughts of hurting yourself?"

[Her Mental Status slides; HA #1 doc sheet: Appearance Jarvis pp. 62–67; Jarvis p. 79]



Her Mental Status slides: Cognitive functions

Cognition: orientation and memory

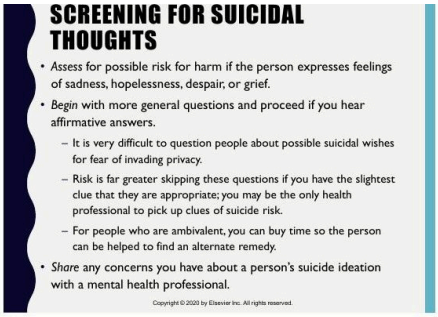
Do: Orientation to time, place, and person. Attention span (do they finish a thought?). Recent memory: 24-hour diet recall. Remote memory: verifiable past events. New learning: the four unrelated words test, with recall at 5, 10, and 30 min.

Technique: Interview.

Normal: Oriented. Under age 60: recalls 3–4 of the 4 words. MMSE 24–30 = no impairment.

> **Say:** "I'm going to say four words. Please remember them, and I'll ask you later: brown, honesty, tulip, eyedropper."

[Her Mental Status slides]



Her Mental Status slides: Screening for suicidal thoughts

Screen for depression and suicide

Do: If they express sadness or hopelessness, use the PHQ-2 (mood and anhedonia), then the PHQ-9 if positive. Start with general questions and get more specific.

Technique: Interview.

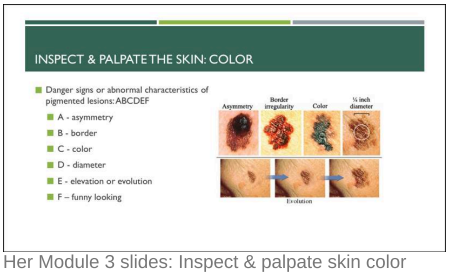
Normal: Denies thoughts of self-harm.

> **Say:** "Over the last 2 weeks, have you felt down or hopeless? Have you ever had thoughts of hurting yourself?"

[Her Mental Status slides]

4. Skin, hair, nails

Check the hands and nails first, then look at the skin with each body region as you go [Jarvis p. 771; Summary Checklist p. 250].



Her Module 3 slides: Inspect & palpate skin color

Inspect and palpate the skin

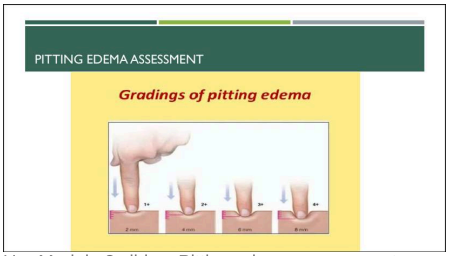
Do: Color, pigmentation, lesions, bruising. Palpate temperature, moisture, texture, thickness, edema, turgor/mobility.

Technique: Inspect, then palpate. Use the backs of your fingers for temperature. Pinch skin under the clavicle for turgor.

Normal: Consistent color for ethnicity, warm, dry, smooth, intact. Turgor recoils right away. No edema or lesions.

> **Say:** "I'm going to look at your skin and feel how warm it is."

[Her slides; doc sheet: Jarvis pp. 207, 209–210]



Her Module 3 slides: Pitting edema assessment

Turgor and edema

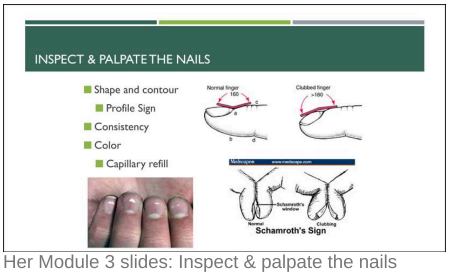
Do: Press over the tibia or ankle for 5 sec. Grade any pitting.

Technique: Palpation.

Normal: No pitting (0). Scale goes 1+ (2 mm) to 4+ (8 mm).

> **Say:** "I'm going to press on your shin for a few seconds. Tell me if it's tender."

[Her slides; doc sheet: edema/pitting Jarvis pp. 210, 519]



Her Module 3 slides: Inspect & palpate the nails

Hair and nails

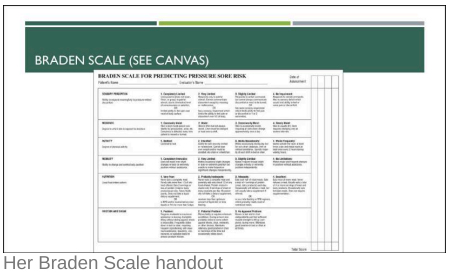
Do: Hair: color, texture, distribution, scalp lesions, lice. Nails: shape and contour (profile angle), consistency, color, capillary refill.

Technique: Inspect. Press the nail bed for capillary refill.

Normal: Nail angle about 160° (no clubbing), pink nail beds, cap refill <2–3 sec, smooth nails. Hair evenly distributed, no lice or nits.

> **Say:** "Can I see your hands? I'm going to squeeze your fingernail for a second."

[Her slides; doc sheet: hair Jarvis p. 213, nails pp. 213–215, cap refill p. 215]



Her Braden Scale handout

Pressure-injury risk

Do: Score the Braden Scale: sensory perception, moisture, activity, mobility, nutrition, friction/shear.
Technique: Scoring tool.
Normal: 19–23 = no risk. 18 or below = at risk.

> **Say:** N/A

[Her Braden handout]

5. HEENT, neck and lymph nodes

Patient sits on the edge of the bed. You stand in front [Jarvis pp. 771–772].

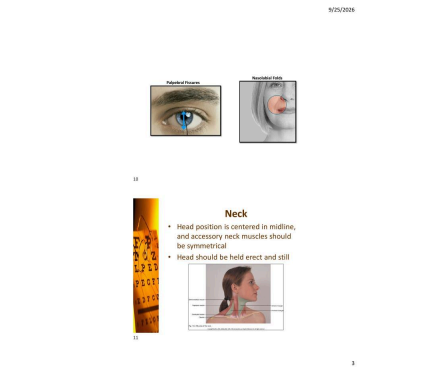

Jarvis p. 771

Head and face

Do: Inspect and palpate scalp, hair, and skull. Look at the face for expression and symmetry (CN VII). Palpate the temporal artery and the TMJ as they open their mouth. Palpate the frontal and maxillary sinuses.
Technique: Inspect, palpate.
Normal: Skull normocephalic and symmetric, face symmetric, TMJ moves smoothly without clicking, sinuses nontender.

> **Say:** "Open and close your mouth for me. Tell me if anything I press on is tender."

[Jarvis p. 771; Summary Checklist p. 279]



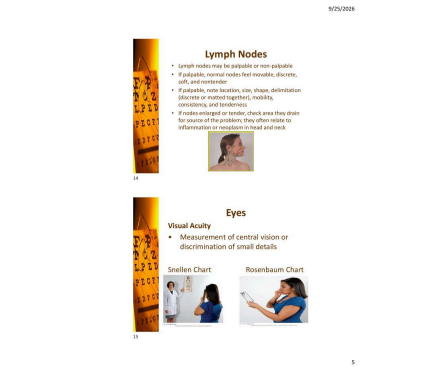
Her Module 2 HEENT slides (eyes)

Eyes

Do: Visual acuity (Snellen). Visual fields by confrontation (CN II). EOMs: corneal light reflex plus the 6 cardinal gazes (CN III, IV, VI). External eye, conjunctiva, sclera, cornea, iris. Pupils: size, light reaction, accommodation.
Technique: Inspection. Use a penlight for pupils: swing it in from the side, then check near vision.
Normal: 20/20. Full fields. EOMs intact with no nystagmus. Sclera white, conjunctiva pink. PERRLA.

> **Say:** "Follow my finger with your eyes only. Keep your head still." "Look at the wall, now look at my finger."

[Her slides; Jarvis p. 771; p. 322]



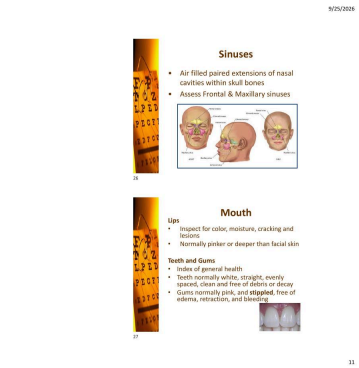
Her Module 2 HEENT slides (ears)

Ears

Do: Inspect the external ear. Move the auricle and press the tragus to check for tenderness. Use the otoscope to view the canal and the TM. Do the whispered voice test.
Technique: Inspection, palpation. Adult otoscope: pull the auricle up and back.
Normal: No tenderness. Canal clear. TM pearly gray with cone of light visible. Repeats whispered words.

> **Say:** "I'm going to look in your ear. Hold still, this won't hurt." "Repeat the words I whisper."

[Her slides; Jarvis p. 772; p. 349]



Her Module 2 HEENT slides (nose/mouth)

Nose, mouth, throat

Do: Nose: symmetry, patency of each nostril, mucosa, septum. Mouth: lips, buccal mucosa, teeth and gums, tongue, floor of mouth, palate. Throat: uvula rises with "ahh" (CN IX, X), grade the tonsils. Tongue out midline (CN XII).
Technique: Inspect with a penlight and tongue blade.
Normal: Nares patent, septum midline. Mucosa pink and moist. Uvula midline and rises. Tonsils 1+ or absent. Tongue midline.

> **Say:** "Press on one nostril and sniff." "Open wide and say 'ahh.'" "Stick out your tongue."

[Her slides; Jarvis p. 772; p. 382]



Jarvis p. 772

Neck and lymph nodes

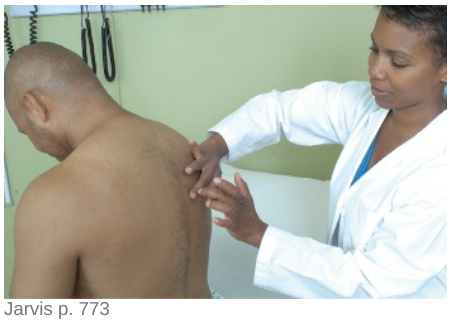
Do: Inspect for symmetry, lumps, and pulsations. Palpate the lymph nodes in order: preauricular, posterior auricular, occipital, submental, submandibular, jugulodigastric, superficial cervical, posterior cervical, deep cervical, supraclavicular. Palpate one carotid at a time. Check the trachea is midline. Test ROM and shoulder shrug (CN XI). From behind, palpate the thyroid.
Technique: Inspect. Palpate gently with finger pads in small circles.
Normal: Nodes not palpable (or small, soft, mobile, nontender). Trachea midline. Thyroid nontender, no nodules. Full ROM, strength 5/5.

> **Say:** "Tip your head slightly toward me." "Shrug your shoulders against my hands." "Swallow for me."

[Jarvis p. 772; p. 279; her Module 2 slides]

6. Chest and lungs

Open the gown in back. Do the posterior chest first, then move to the front [Jarvis p. 773].



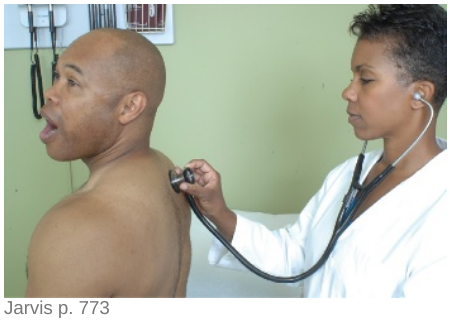
Jarvis p. 773

Posterior chest: inspect and palpate

Do: Inspect the shape of the chest, the skin, and symmetry. Palpate symmetric expansion and tactile fremitus ("ninety-nine"), plus any lumps or tenderness.
Technique: Inspect, then palpate.
Normal: AP:lateral ratio about 1:2. Expansion symmetric. Fremitus equal on both sides.

> **Say:** "Take a deep breath." "Say 'ninety-nine' each time you feel my hand."

[Jarvis p. 773; p. 453]



Jarvis p. 773

Posterior chest: percuss and auscultate

Do: Percuss the lung fields and the CVA. Auscultate side to side, top to bottom, with the diaphragm, one full breath per spot.
Technique: Percussion, then auscultation in a ladder pattern, comparing sides.
Normal: Resonant. No CVA tenderness. Vesicular sounds over the peripheral fields, no adventitious sounds (crackles, wheezes).

> **Say:** "Breathe in and out through your mouth, a little deeper than normal."

[Jarvis p. 773]


Jarvis p. 773

Anterior chest

Do: Inspect respirations and skin. Palpate expansion and fremitus. Percuss and auscultate the anterior and lateral fields, comparing sides.
Technique: Inspect, palpate, percuss, auscultate.
Normal: Relaxed, even respirations at 12–20. Clear breath sounds bilaterally.

> **Say:** "Keep breathing through your mouth for me."

[Jarvis p. 773; Chest/lungs slides not on file (gap)]

7. Heart, neck vessels, peripheral vascular

Upper extremities are done sitting. Heart is done supine at 30°. Legs are done supine [Jarvis pp. 773–776].


Jarvis p. 773

Upper extremities (PVS)

Do: Test ROM and strength of the hands, wrists, arms, and shoulders. Palpate the epitrochlear nodes. Check temperature and capillary refill. Compare radial and brachial pulses on both sides.
Technique: Inspect, palpate.
Normal: Warm, cap refill <2–3 sec, pulses 2+ and equal.

> **Say:** "Squeeze my fingers as hard as you can."

[Jarvis p. 773; p. 532]

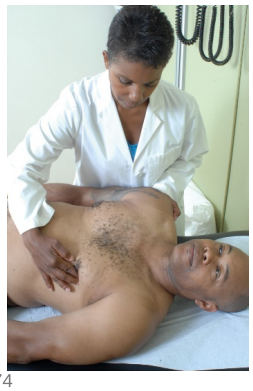

Jarvis p. 774

Neck vessels and precordium

Do: Patient lies supine at 30°. Look for the jugular venous pulse. Inspect and palpate the precordium. Find the apical impulse (5th ICS, MCL).
Technique: Inspect, palpate.
Normal: No JVD at 45°. Apical impulse at the 5th ICS, MCL. No heaves or thrills.

> **Say:** "Turn your head slightly to the left for me."

[Jarvis p. 774; p. 504]


Jarvis p. 774

Auscultate the heart

Do: Count the apical rate and rhythm for 1 full minute. Listen at the aortic (2nd R ICS), pulmonic (2nd L ICS), Erb's point (3rd L), tricuspid (4th L LSB), and mitral/apex (5th MCL) areas. Use the diaphragm, then the bell. Roll them to the left side and listen at the apex with the bell.
Technique: Auscultate in a Z pattern.
Normal: Regular rate and rhythm, S1 and S2 heard, no murmurs, gallops, or rubs. S1 is loudest at the apex, S2 at the base.

> **Say:** "Breathe normally. I'm going to listen to your heart in a few spots."

[Jarvis p. 774; heart slides not on file (gap)]



Jarvis p. 776

Lower extremities (PVS)

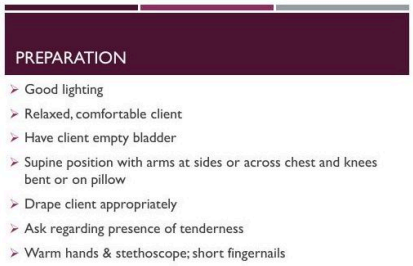
Do: Inspect for symmetry, skin, and hair distribution. Palpate the femoral, popliteal, posterior tibial, and dorsalis pedis pulses. Check temperature and pretibial edema. Look between the toes.
Technique: Inspect, palpate.
Normal: Warm, pulses 2+ and equal, no edema, hair evenly distributed, no lesions.

> **Say:** "I'm going to feel the pulses in your feet."

[Jarvis p. 776]

8. Abdomen: inspect, auscultate, percuss, palpate

Patient lies supine and flat with knees slightly bent. Drape from chest to pubis. You auscultate before you percuss or palpate, so you don't change the bowel sounds [Jarvis p. 775; her Module 4 slides].



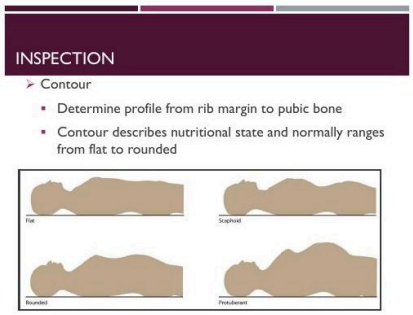
Her Module 4 slides: Preparation

Prep for the abdomen

Do: Good lighting. Patient empties bladder. Supine with arms at sides or across chest, knees bent or on a pillow. Drape. Ask about tenderness. Warm your hands and stethoscope. Keep nails short.
Technique: N/A
Normal: Relaxed patient.

> **Say:** "Do you have pain anywhere in your belly? I'll warm my hands first."

[Her Module 4 slides]



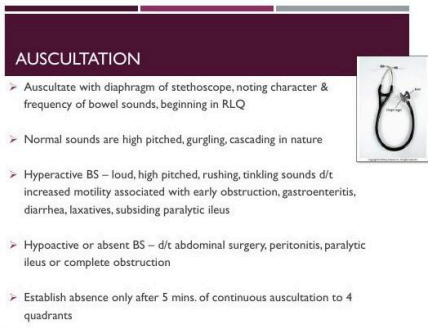
Her Module 4 slides: Inspection

Inspect

Do: Look at contour, symmetry, umbilicus, skin (striae, scars), pulsations, and movement.
Technique: Inspection, with the patient's bladder empty.
Normal: Flat or rounded, symmetric, umbilicus midline and inverted, no visible peristalsis.

> **Say:** "Lie flat with your arms at your sides. I'm going to look at your belly first."

[Her slides; doc sheet: Jarvis p. 542]



Her Module 4 slides: Auscultation

Auscultate

Do: Bowel sounds in all 4 quadrants using the diaphragm, **starting in the RLQ**. Only call them absent after 5 min of continuous listening. Use the bell over the aorta, renal, iliac, and femoral arteries to listen for bruits.
Technique: Auscultation.
Normal: High-pitched, gurgling, cascading sounds. Hyperactive = rushing or tinkling. Hypoactive or absent = post-op, ileus. No bruits.

> **Say:** "I'm going to listen to your belly for a minute. Breathe normally."

[Her slides; doc sheet: Jarvis p. 545]



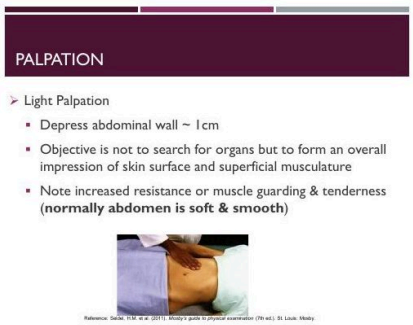
Her Module 4 slides: Percussion

Percuss

Do: Her slides: percussion is **NOT** recommended for liver span, spleen, or bladder. Percuss the **costovertebral angle** for kidney tenderness. (Jarvis also has you percuss all 4 quadrants for tympany. Ask her if she wants that.)
Technique: Indirect fist percussion at the CVA.
Normal: No CVA tenderness. Tympany over most of the abdomen.

> **Say:** "You'll feel a thump on your back. Tell me if it hurts."

[Her slides; doc sheet: Jarvis p. 546]



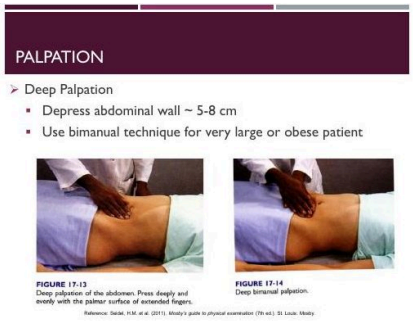
Her Module 4 slides: Light palpation (~1 cm)

Palpate: light, then deep

Do: Light palpation about 1 cm deep in all quadrants. Then deep palpation 5–8 cm with one hand or two. Check rebound tenderness only if indicated. Palpate the liver, spleen, and kidneys. Do tender areas last.
Technique: Palpation.
Normal: Soft, nontender, no masses, no guarding.

> **Say:** "Let me know if anything is tender. I'll press tender spots last."

[Her slides; doc sheet: Jarvis p. 548]



14

Her Module 4 slides: Deep palpation (5–8 cm)

Deep palpation

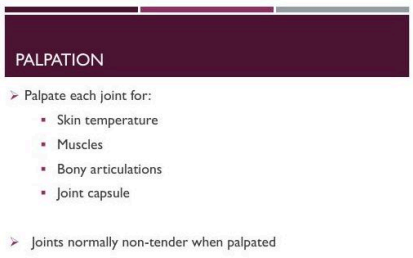
Do: Use the bimanual technique for large or obese abdomens.
Technique: Palpation.
Normal: No organomegaly or masses.

> **Say:** "Take a deep breath in."

[Her slides]

9. Musculoskeletal

For each joint: inspect, palpate, ROM, then strength [Jarvis p. 627; her Module 4 slides].



Her Module 4 slides: Palpation

Palpate joints

Do: Palpate each joint for skin temperature, muscles, bony articulations, and the joint capsule.
Technique: Inspection: size, contour, skin. Then palpation.
Normal: Nontender, no swelling, symmetric.

> **Say:** "Tell me if any of this is sore."

[Her slides; doc sheet: Jarvis pp. 585–586]



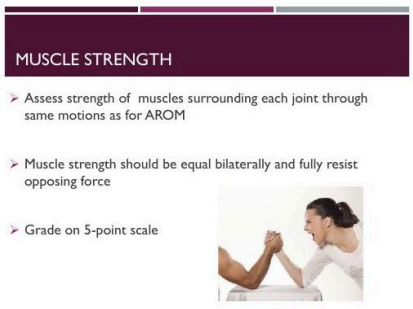
Her Module 4 slides: ROM, head-to-toe approach

Range of motion

Do: Active ROM head to toe: TMJ, cervical spine, shoulder, elbow, wrist/hand, hip, knee, ankle/foot, spine. Do passive ROM only if active is limited. Use a goniometer if abnormal.
Technique: Inspect, palpate, ROM.
Normal: Full ROM, no pain or crepitus, joints symmetric with no swelling.

> **Say:** "Copy what I do. Raise your arms over your head..."

[Her slides; Jarvis p. 627]



Her Module 4 slides: Muscle strength

Muscle strength

Do: Have them push and pull against your resistance through the ROM, comparing both sides. Grade it on the 0–5 scale.
Technique: Palpation/resistance.
Normal: 5/5, equal on both sides, fully resists.

> **Say:** "Push against my hand. Don't let me move you."

[Her slides; doc sheet: grading scale Jarvis p. 587]



Jarvis p. 778

Gait, balance, spine

Do: Patient stands (gown on, stay close). Walk and turn, heel-to-toe, toe-walk and heel-walk, Romberg, shallow knee bend. From behind, check the spine as they touch their toes.
Technique: Inspection, standing.
Normal: Smooth, coordinated gait. Negative Romberg. Spine straight with no scoliosis.

> **Say:** "Walk to the door and back, heel to toe. I'm right here."

[Jarvis p. 778]

10. Neuro basics

Patient sits with legs dangling [Jarvis pp. 776–777; Summary Checklist p. 684].



Jarvis p. 777

Sensation and cerebellar

Do: Light touch and sharp/dull on the face, arms, and legs. Position sense and stereognosis. Finger-to-nose, rapid alternating movements, and heel-to-shin.
Technique: Inspection.
Normal: Sensation intact bilaterally. Movements smooth and accurate.

> **Say:** "Close your eyes. Tell me 'sharp' or 'dull.'" "Touch my finger, then your nose."

[Jarvis pp. 776–777]


Jarvis p. 777

Reflexes (if covered in lab)

Do: Biceps, triceps, brachioradialis, patellar, and Achilles reflexes, plus Babinski.
Technique: Reflex hammer.
Normal: 2+ and symmetric. Babinski negative (toes curl down).

> **Say:** "Let your leg hang loose."

[Jarvis p. 777; ask her if reflexes are tested in check-off (gap)]

11. Closing

[Jarvis p. 779]


Jarvis p. 775

Safety, call light, hand hygiene, document

Do: Help them get comfortable and dressed. Bed low and locked, side rails per policy, call light and phone within reach. Ask if they have questions and thank them. Do hand hygiene. Document right away: normal findings plus anything abnormal ("if it isn't documented, it wasn't done").
Technique: N/A
Normal: Patient is safe, comfortable, and has the call light.

> **Say:** "We're all done. Here's your call light. Is there anything else I can get you?"

[Jarvis pp. 779–780]

One-page printable checklist

Prep (15 min limit, no prompting)

- ☐ Equipment ready
- ☐ Hand hygiene
- ☐ Introduce self/role
- ☐ 2 identifiers
- ☐ Privacy
- ☐ Explain + consent

General survey/VS

- ☐ Appearance, LOC, gait, hygiene
- ☐ Ht/Wt/BMI
- ☐ Pulse, RR, BP, Temp, SpO2
- ☐ Pain (PQRST)

Mental status

- ☐ Appearance/behavior
- ☐ A&O ×4
- ☐ Memory: 4 words
- ☐ Mood/affect, thought, PHQ-2/suicide

Skin/hair/nails

- ☐ Color, temp, moisture
- ☐ Turgor, edema
- ☐ Lesions
- ☐ Hair/scalp
- ☐ Nails, cap refill
- ☐ Braden

HEENT/neck

- ☐ Skull, face (CN VII), TMJ, sinuses
- ☐ Snellen, fields (II), EOMs (III,IV,VI)
- ☐ PERRLA
- ☐ Ear: tragus, otoscope, whisper
- ☐ Nose patency
- ☐ Mouth, uvula (IX,X), tongue (XII)
- ☐ Lymph nodes ×10
- ☐ Carotid one side, trachea
- ☐ ROM, shrug (XI)
- ☐ Thyroid (posterior)

Chest/lungs

- ☐ Inspect shape, resp
- ☐ Expansion, fremitus
- ☐ Percuss, CVA
- ☐ Auscultate posterior
- ☐ Anterior chest

Heart/PVS

- ☐ Arms: ROM, cap refill, radial/brachial
- ☐ JVP
- ☐ Apical impulse 5th ICS MCL
- ☐ Apical HR 1 min
- ☐ A-P-E-T-M diaphragm + bell
- ☐ Legs: pulses, edema, toes

Abdomen

- ☐ Inspect
- ☐ Auscultate from RLQ ×4 + bruits
- ☐ Light palpation
- ☐ Deep palpation
- ☐ CVA tenderness

MSK

- ☐ Palpate joints
- ☐ ROM head-to-toe bilateral
- ☐ Strength 0–5
- ☐ Gait, Romberg, spine

Neuro

- ☐ Sensation
- ☐ Finger-nose, RAM, heel-shin
- ☐ Reflexes (if req.)

Closing

- ☐ Comfort, bed low/locked
- ☐ Call light in reach
- ☐ Questions, thank
- ☐ Hand hygiene
- ☐ Document